

SUBJECTIVE:

- * Patient reports involvement in a motor vehicle accident approximately 24 hours ago. The patient describes it as a rear-end collision.
- * Reports immediate onset of neck pain, stiffness, and a dull headache radiating to the temples.
- * Also notes generalized low back pain and bilateral shoulder discomfort.
- * Patient denies loss of consciousness at the scene of the accident.
- * Reports wearing a seatbelt at the time of impact; airbags deployed.
- * Rates current neck pain as 7/10 on a scale of 0-10, constant, exacerbated by movement. Back pain is 6/10, aching.
- * Denies any focal numbness, tingling, or weakness in the extremities at this time.
- * Reports feeling "shaken up" and experiencing mild dizziness intermittently since the accident.
- * No significant past medical history provided.

OBJECTIVE:

- * Expected physical exam findings:

- * General: Alert and oriented x3, appears uncomfortable, guarding movements of the head and neck.
- * HEENT: Normocephalic, pupils equal, round, and reactive to light. Tenderness to palpation along the cervical paraspinal muscles. Limited and painful range of motion of the cervical spine (flexion, extension, rotation, lateral bending).
- * Back: Tenderness to palpation over the lumbar paraspinal muscles. Muscle spasms noted in the lumbar region. Painful and limited range of motion of the thoracolumbar spine.
- * Extremities: No gross deformities, swelling, or ecchymosis. Full range of motion in upper and lower extremities, though shoulder movement may be tender with cervical and thoracic involvement. Distal pulses intact.
- * Neurological: Grossly intact motor strength (5/5 throughout). Sensation intact to light touch. Deep tendon reflexes symmetrical. No pathological reflexes elicited.
- * Vital signs: Unavailable.

ASSESSMENT:

- * Primary Diagnosis:

- * Cervical Sprain/Strain (Whiplash Associated Disorder)
- * Lumbar Sprain/Strain
- * Headache, post-traumatic
- * Muscle spasms, acute

- * Differential Diagnosis:

- * Cervical, thoracic, or lumbar fracture
- * Concussion
- * Herniated intervertebral disc (cervical or lumbar)
- * Ligamentous instability of the cervical or lumbar spine
- * Rib fractures (if chest wall pain present)
- * Internal organ injury (e.g., splenic injury, pneumothorax) - less likely given current symptoms but should be considered in high-impact MVC.

PLAN:

- * Tests:

- * X-rays of the cervical spine, thoracic spine, and lumbar spine to rule out acute fracture or dislocation.
- * Consider a CT scan of the head if headache worsens, dizziness persists, or if there was any transient loss of consciousness.

- * Medications:

- * Initiate NSAIDs (e.g., ibuprofen 600mg orally every 8 hours with food) for pain and inflammation.
- * Prescribe a muscle relaxant (e.g., cyclobenzaprine 5mg orally at bedtime) for muscle spasms.
- * Recommend Acetaminophen for breakthrough pain.

- * Patient Advice:

- * Apply ice packs to affected areas for 15-20 minutes, 3-4 times daily for the first 48-72 hours, then transition to heat.
- * Encourage gentle range of motion exercises for the neck and back as tolerated. Avoid strenuous activities, heavy lifting, and prolonged static positions.

- * Monitor for worsening symptoms such as increased pain, numbness, tingling, weakness, vision changes, or severe headaches, and seek immediate medical attention if they occur.
- * Maintain good posture and use supportive pillows.
- * Follow-up Instructions:
 - * Return to clinic in 1-2 weeks for re-evaluation of symptoms and progress.
 - * Referral to physical therapy for comprehensive rehabilitation of cervical and lumbar spine.
 - * Consider referral to neurology if concussion symptoms persist or worsen.